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Implementing Quality Improvement for Psychosocial Interventions

To the Editor A Viewpoint by Drs Pincus and England summarized a new report by the Institute of Medicine on improving the quality of psychosocial interventions for mental and substance use disorders.¹ We wish to draw attention to advances in intervention science that could facilitate implementation of recommendations in the report.

The Behavior Change Technique Taxonomy addresses the recommendation for a common terminology for specifying elements of psychosocial intervention elements.² This taxonomy was developed using consensus methods and includes a hierarchically structured catalog of 93 distinct intervention elements. Use of this taxonomy would improve the clarity and synthesis of research. Widespread adoption could further the goals of identifying key elements within and across interventions, discovering optimal dosing and sequencing of elements, and connecting elements to purported moderators or mechanisms.

New methods for research synthesis can facilitate implementation of the recommendations regarding high-quality systematic reviews of psychosocial intervention elements.³ Theory-based meta-regression involves using theoretical frameworks to select intervention elements for meta-analytic investigation. Individual participant-level data meta-analysis is much more powerful than meta-analysis using aggregate data for identifying mediators and moderators of intervention effects. Network meta-analysis can use all relevant trials in one analysis to probabilistically rank multiple interventions according to their comparative effectiveness. Qualitative meta-syntheses offer an explicit and transparent method for developing robust descriptive and analytic themes about intervention implementation across a body of literature. Greater use of these research synthesis techniques can inform clinical guidelines and implementation tools on psychosocial intervention elements.

The report recommends developing performance measures to guide continuous quality improvement for psychosocial interventions. As it notes, standardization is limited because consensus is often lacking about which outcomes and measures should be prioritized. The Core Outcome Measures in Effectiveness Trials Initiative aims to further the development of “core outcome sets”—based on research evidence and stakeholder consensus—that specify the most important outcomes to measure when evaluating interventions for a specific condition and the best extant tools to measure these outcomes.⁴ Although originally intended for use in clinical trials, core outcome sets are increasingly used to select outcomes for clinical audit and performance measurement—and could be developed for psychosocial

interventions targeting mental health and substance use disorders for this purpose.

Such advances may be useful to implementation of the report’s recommendations and may facilitate the goal of improving scientific evidence and quality improvement for psychosocial interventions.

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Conflict of Interest Disclosures: The authors have completed and submitted the ICMJE Form for Disclosure of Potential Conflicts of Interest and reported being investigators on the Consolidated Standards of Reporting Trials for Social and Psychological Interventions (CONSORT-SPI).

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In Reply We agree with Dr Grant and colleagues that the advances in implementation science they note have the potential to make an important contribution toward implementation of the recommendations of the Institute of Medicine to improve the quality of psychosocial interventions for mental and substance use disorders. In particular, their comments support the recommendation for “developing a common terminology for describing and classifying the elements of evidence-based psychosocial interventions,” as well as recommendations related to evidence synthesis and future research needs. Furthermore, we believe that the Behavioral Change Technique Taxonomy is a useful effort toward developing an ontology of intervention elements that would not only be useful for research on psychosocial interventions but also lay the groundwork for advancing needed quality measurement and improvement strategies for mental illness and substance abuse care.

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Conflict of Interest Disclosures: The authors have completed and submitted the ICMJE Form for Disclosure of Potential Conflicts of Interest. Dr Pincus reported receiving grants from the Commonwealth Fund; National Center for Advancing Translational Sciences; National Institute on Alcohol Abuse and Alcoholism; Department of Health and Human Services/Substance Abuse and Mental Health Services Administration/Office of the Assistant Secretary for Planning and Evaluation; Atlantic Philanthropies; John A. Hartford Foundation; the governments of Australia, Canada, England, Germany, Ireland, the Netherlands, New Zealand, Norway, and Scotland; New York State Health Foundation; Patient-Centered Outcomes Research Institute; and Department of Defense; receiving personal fees from RAND Corporation, Mathematica Policy Research, and Manila Consulting; and receiving travel support to attend committee meetings of the National Quality Forum, National Committee for Quality Assurance, and the American Psychiatric Association. No other disclosures were reported.

CORRECTION

Coordinating Clinical Site Omitted: In the Original Investigation entitled "Panretinal Photocoagulation vs Intravitreal Ranibizumab for Proliferative Diabetic Retinopathy: A Randomized Clinical Trial" published in the November 24, 2015, issue of *JAMA*,¹ a coordinating clinical site was omitted from eAppendix 1 in Supplement 1: Grand Blanc, MI Retina Vitreous Center (4): Robin D. Ross (I); Susan M. Sanford (C); Nicole Martini Markiewicz (C); Tracy M. Utley (C,V); Shannon Henderson (V); Mary D. Walker (V); Joanie H. Lippincott (P); Patricia Streasick (P). This article was corrected online.

1. Gross JG, Glassman AR, Jampol LM, et al; Writing Committee for the Diabetic Retinopathy Clinical Research Network. Panretinal photocoagulation vs intravitreal ranibizumab for proliferative diabetic retinopathy: a randomized clinical trial. *JAMA*. 2015;314(20):2137-2146.

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